

ENGLISH FOR WORK / SEPTEMBER 2026

Nursing and Allied Health English



Conversation lab

8 complete workplace dialogues, followed by eight additional role-play cases. Study the exchanges, then make the conversation your own.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For nurses, charge nurses, physical therapists, occupational therapists, respiratory therapists, imaging staff, laboratory staff, care-team coordinators, and allied-health supervisors.

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Inside this conversation lab

Original fictional training conversations in professional English. These are realistic scripts, not transcripts of actual people. The professional roles are the speaker labels; all figures, incidents, and organizations are invented.

01 Shift handoff with a pending result

02 Escalating a change

03 Medication reconciliation discrepancy

04 Preparing teach-back

05 Interprofessional rounds

06 Correcting a chart entry

07 Family communication boundaries

08 A near-miss debrief

Then try eight independent role-play cases, followed by feedback criteria and references.

Read it. Hear it. Make it yours.

1. Read the setting. Identify the roles, the immediate problem, and what each speaker needs.
2. Read the whole conversation aloud with a partner. In a group, give a third person the observer role. For three-speaker scripts, assign each professional a separate voice.
3. Notice how the speakers ask a specific question, qualify a claim, disagree, explain a technical point, or agree on a next step. Underline language you would actually use.
4. Cover the script. Recreate the exchange using the situation and professional roles. Keep the meaning, but change the wording.
5. Switch roles and introduce a complication: a missing record, a different priority, an unavailable colleague, or a changed assumption. End with a clear outcome or unresolved question.

Register and terminology

These colleagues use concise professional language. In an internal technical meeting, familiar abbreviations can be natural. With a new colleague or a client, expand unfamiliar terms and check understanding. Keep disagreement directed at the evidence or proposal, and match the degree of certainty to what is known.

Independent study

Speak each role aloud. Pause before the reply and supply your own response before revealing the next line. Record a second version using invented details, then compare its clarity and tone with the script.

Professional context

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

FULL DIALOGUE 01 / 6 SPEAKING TURNS

Shift handoff with a pending result

Two nurses transfer care using the approved handoff process.

Roles: Outgoing nurse / Incoming nurse

Outgoing nurse: For the patient we've identified in the chart, the repeat lab result is still pending. The care team has been notified.

Incoming nurse: What prompted the repeat, and what follow-up has been requested?

Outgoing nurse: The initial sample was unsuitable. The record includes the collection time and the clinician's current plan.

Incoming nurse: I'll verify that plan in the chart. Who is checking for the new result?

Outgoing nurse: I'm handing that follow-up to you now. Please acknowledge it in our handoff record.

Incoming nurse: Understood. I'll check the result and escalate according to the documented plan and local procedure.

Notice the professional language

Discuss this wording: "What prompted the repeat, and what follow-up has been requested?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 02 / 6 SPEAKING TURNS

Escalating a change

A nurse reports a patient's change in condition.

Roles: Staff nurse / Charge nurse

Staff nurse: I'm concerned about a change from the patient's earlier assessment and need a prompt clinical review.

Charge nurse: Give me the situation and relevant background, then the findings you've observed.

Staff nurse: The patient is newly confused compared with this morning. I've recorded the current observations and followed our escalation process.

Charge nurse: Who has been contacted, and have they acknowledged the request?

Staff nurse: The responsible clinician has been contacted; I'm still waiting for acknowledgment.

Charge nurse: I'll support the escalation now under our local pathway. Keep the observations and response times clearly documented.

Notice the professional language

Discuss this wording: "Who has been contacted, and have they acknowledged the request?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 03 / 6 SPEAKING TURNS

Medication reconciliation discrepancy

A nurse and pharmacist clarify inconsistent records.

Roles: Ward nurse / Pharmacist

Ward nurse: The admission list and medication chart show different doses. I haven't treated either list as confirmed.

Pharmacist: What sources are available for reconciliation?

Ward nurse: The transfer document and the patient's reported home list. The prescriber still needs to resolve the discrepancy.

Pharmacist: I'll review those sources and contact the prescribing team through our process.

Ward nurse: The allergy entry is also incomplete. I'll flag that rather than leave it implicit.

Pharmacist: Good. We'll document the clarification and communicate the authorized plan before administration decisions proceed.

Notice the professional language

Discuss this wording: "What sources are available for reconciliation?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 04 / 6 SPEAKING TURNS

Preparing teach-back

A nurse and therapist coordinate discharge education.

Roles: Nurse / Occupational therapist

Nurse: The patient nodded during the equipment explanation, but we haven't checked their understanding.

Occupational therapist: Let's ask them to show how they would use it, with the approved instructions in front of them.

Nurse: I'll explain that we're checking how well we taught it, not testing them.

Occupational therapist: Use their preferred language and arrange a qualified interpreter if needed.

Nurse: If the demonstration shows a gap, we'll explain that part again and repeat the check.

Occupational therapist: I'll document the teaching, the observed response, and any further support required for discharge planning.

Notice the professional language

Discuss this wording: "The patient nodded during the equipment explanation, but we haven't checked their understanding." What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 05 / 6 SPEAKING TURNS

Interprofessional rounds

A therapist and discharge coordinator discuss readiness.

Roles: Physical therapist / Discharge coordinator

Physical therapist: Mobility assessment is complete, but the home access information is still missing.

Discharge coordinator: The family mentioned stairs. Can we record the patient as discharge-ready?

Physical therapist: My assessment addresses observed mobility here. It doesn't establish that the home setup is suitable.

Discharge coordinator: I'll obtain the access details and bring the outstanding issue to the team.

Physical therapist: I'll explain the functional findings and any assessment limitations at rounds.

Discharge coordinator: Then the responsible team can confirm the discharge plan with those factors in view.

Notice the professional language

Discuss this wording: "The family mentioned stairs. Can we record the patient as discharge-ready?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 06 / 6 SPEAKING TURNS

Correcting a chart entry

A nurse asks about an inaccurate documentation time.

Roles: Staff nurse / Charge nurse

Staff nurse: I entered the wrong observation time. The recorded value is correct, but the timestamp isn't.

Charge nurse: Use the approved correction process so the original entry and correction remain traceable.

Staff nurse: I shouldn't delete the entry and recreate it as though it were written earlier?

Charge nurse: Correct. State what is being corrected and follow the documentation policy.

Staff nurse: I'll also notify the clinician if the timing affects their interpretation.

Charge nurse: Yes. Accurate documentation and timely communication both matter; correcting one doesn't replace the other.

Notice the professional language

Discuss this wording: "I shouldn't delete the entry and recreate it as though it were written earlier?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 07 / 6 SPEAKING TURNS

Family communication boundaries

Two care-team members plan a response to a distressed relative.

Roles: Ward nurse / Patient liaison

Ward nurse: A caller wants detailed results, but I haven't verified their identity or permission to receive information.

Patient liaison: Acknowledge their concern and follow the consent and identity checks before any disclosure.

Ward nurse: They feel we're refusing to help. How can I explain the next step clearly?

Patient liaison: Say that you're checking how the team can share information appropriately, and identify who will follow up.

Ward nurse: I'll ask the responsible team to clarify the communication arrangement with the patient.

Patient liaison: I'll support the conversation and document the agreed contact route through the approved process.

Notice the professional language

Discuss this wording: "They feel we're refusing to help. How can I explain the next step clearly?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 08 / 6 SPEAKING TURNS

A near-miss debrief

A nurse and safety lead review a specimen-labeling near miss.

Roles: Nurse / Safety lead

Nurse: I noticed the label mismatch before the specimen left the ward. I followed the specimen procedure and reported it.

Safety lead: Let's walk through the sequence without assuming why it happened.

Nurse: Two requests were open at once. The label printed beside another patient's paperwork.

Safety lead: What made the mismatch visible, and which checks helped you catch it?

Nurse: The identity check. I'd like us to review how printouts are separated at the workstation.

Safety lead: We'll examine the workflow and record improvement actions without replacing the formal incident review.

Notice the professional language

Discuss this wording: "What made the mismatch visible, and which checks helped you catch it?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

Eight more situations to make your own

The following cases are open role plays. They give you facts, contrasting roles, useful expressions, and a short model response. Use what you learned from the complete dialogues to create a longer exchange.

Preparation: two minutes. Conversation: three to five minutes. Feedback: one minute. Switch roles and repeat with the second-round challenge.

ROLE-PLAY CASE 01

Shift Handoffs and Clinical Prioritization

SHARED CASE

In a simulated handoff, the outgoing clinician has observations and background notes, but a requested result is still pending. The receiving clinician needs a clear transfer of information.

Role A | Responding professional

Give a handoff that the receiving person can confirm. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are taking over the work. Ask what remains open and repeat back the critical status or responsibility. Point out one detail that would be unsafe or misleading to assume.

Useful expressions

The current status is ...; the outstanding item is

The record shows

Please confirm who has accepted

Cover this until after your first attempt

The result is still pending. I'll separate the current situation, relevant background, my assessment, and the request in this handoff. Please repeat back the outstanding task so we can confirm understanding.

Second round

The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 02

Patient Assessment and Escalation

SHARED CASE

During a simulation, a patient reports feeling worse and the learner identifies a change from the earlier observation. The local escalation process is available.

Role A | Responding professional

Make a request with a clear action, purpose, and realistic timing. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You can help but need a clear request and its purpose. Ask what is required and whether the timing is fixed or negotiable.

Useful expressions

Could you provide ... so that ...?

Please confirm ... by [agreed time].

If that timing is not possible, please let me know

Cover this until after your first attempt

I'm concerned because this is a change from the earlier observation. I need an assessment through our escalation process now. Here is what I observed and when it changed.

Second round

The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 03

Medication Safety and Allergy Clarification

SHARED CASE

A fictional medication record and the patient's account contain different allergy information. The learner must communicate the discrepancy through the clinical process.

Role A | Responding professional

Clarify missing information before responding. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You need to know exactly what information is missing. Give one answer only after your partner asks a focused question; then ask them to confirm their understanding.

Useful expressions

When you say ..., do you mean ...?

Could you clarify which ...?

So, my understanding is Is that right?

Cover this until after your first attempt

The recorded allergy information differs from what the patient has told me. Could we verify the details through the appropriate clinical process? I will state the discrepancy clearly rather than assume either account is complete.

Second round

The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 04

Patient Education and Teach-Back

SHARED CASE

A patient in a role-play nods during an explanation of an approved discharge sheet. The learner needs to check whether the explanation was clear.

Role A | Responding professional

Explain a useful distinction in plain English. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are unfamiliar with one technical term in the case. Ask for a plain-English explanation and then paraphrase it. Do not pretend to understand a term you cannot explain.

Useful expressions

In this context, ... means

The difference is that

How would you explain that distinction to a colleague?

Cover this until after your first attempt

I want to check that I explained this clearly. In your own words, how will you follow the instructions when you get home? We can go over any part that is unclear.

Second round

Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 05

Interprofessional Rounds

SHARED CASE

In simulated rounds, one team member focuses on discharge timing and another raises an unresolved mobility concern.

Role A | Responding professional

Include relevant voices and clarify the decision process. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You hold a relevant concern that has not been discussed. Raise it when invited and ask who will decide. Help the facilitator state one unresolved question accurately.

Useful expressions

Let's hear ... before we decide.

The point we still need to resolve is

Who will make this decision, and what do they need?

Cover this until after your first attempt

Before we agree on timing, could we hear the mobility concern and the current assessment? Let's confirm the outstanding questions and who will address them in the care plan.

Second round

A participant interrupts or the meeting is running out of time. Protect a turn to speak and close with an accurate decision record.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 06

Documentation and Charting

SHARED CASE

A draft simulated note says a patient was "difficult." It contains no description of what was said or observed.

Role A | Responding professional

Separate observations, assumptions, and conclusions. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Useful expressions

The available evidence shows

This may indicate ..., but

We have not yet established whether

Cover this until after your first attempt

Let's replace that label with observable information. Record what the patient said or did, the relevant context, the action taken, and the response, following the documentation process.

Second round

Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 07

Difficult Families and Boundaries

SHARED CASE

A family member in a role-play says, "Nobody tells us anything." The learner can explain their role and arrange an appropriate conversation.

Role A | Responding professional

Acknowledge a communication problem and give a corrected message. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are frustrated because an earlier message created an expectation. Explain that expectation. Ask your partner to distinguish the correction from anything still uncertain.

Useful expressions

My earlier message did not make ... clear.

I'm sorry for The correct information is

Let me check that the revised explanation addresses

Cover this until after your first attempt

I can hear that the lack of information is frustrating. Let me explain what I can clarify and arrange contact with the appropriate team member for the questions outside my role.

Second round

The listener remains disappointed after the correction. Acknowledge the impact and restate the available next step calmly.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 08

Safety Events and Just Culture

SHARED CASE

A simulated safety report includes a fall and an assumption that a colleague was careless. The review has not established the contributing factors.

Role A | Responding professional

Separate observations, assumptions, and conclusions. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Useful expressions

The available evidence shows

This may indicate ..., but

We have not yet established whether

Cover this until after your first attempt

The report should describe the event and available observations. We have not established the contributing factors. I'll remove the unsupported judgment and use the reporting process to raise the concern.

Second round

Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

Lesson conversations and speaking practice

The following sections match 05 and 06 on the industry webpage and in the learner workbook. Each lesson has three ten-turn conversations and two bounded speaking scenarios. These are original fictional teaching scripts, not transcripts of real people.

LESSON 01 / 05 • CONVERSATIONS / 1 OF 3

An SBAR handoff with a pending result

Fictional clinical simulation: case S4 reported tiredness at 14:00 and answered questions while seated. A blood count requested at 13:00 has no result displayed; sample receipt is unknown. Diagnosis and treatment remain outside this language exercise.

01 • Outgoing nurse: Using SBAR for S4: at fourteen hundred, the patient reported tiredness and answered my questions while seated.

02 • Incoming nurse: What background and pending information do I need before accepting the handoff?

03 • Outgoing nurse: A blood count was requested at thirteen hundred. No result is displayed; I haven't confirmed sample receipt.

04 • Incoming nurse: So pending lab doesn't mean you've verified that the laboratory has the sample?

05 • Outgoing nurse: Correct. I observed the conversation, but I haven't established the cause of the reported tiredness.

06 • Incoming nurse: What follow-up are you asking me to take ownership of at this handoff?

07 • Outgoing nurse: Please verify the sample and result status through our clinical process and confirm the appropriate follow-up.

08 • Incoming nurse: I'll own that check; tiredness was reported, questions were answered, and the blood-count status remains unresolved.

09 • Outgoing nurse: That's an accurate read-back. I haven't inferred a diagnosis from the pending result.

10 • Incoming nurse: Handoff accepted. I'll communicate the verified status and follow the local process for concerns.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 2 OF 3

Clarifying the watcher label at shift change

Separate fictional handoff exercise: the training board labels case W3 a watcher, but the reason and current review status are not included.

01 • Receiving nurse: Case W3 is labeled watcher. What concern does that label communicate in this simulation?

02 • Shift coordinator: The board doesn't include the reason, so I need to verify it with the responsible clinician.

03 • Receiving nurse: I don't want to infer acuity or a monitoring plan from the label alone.

04 • Shift coordinator: Agreed. We need the actual concern and the current plan, not just shorthand.

05 • Receiving nurse: Could you obtain that clarification before responsibility is treated as fully transferred?

06 • Shift coordinator: Yes. I'll remain responsible for resolving this handoff gap and identify the responding clinician.

07 • Receiving nurse: Please keep any pending lab or review separate from completed assessments in the update.

08 • Shift coordinator: I will. Nothing in the board entry confirms that those activities have happened.

09 • Receiving nurse: I'll review the information once you return and read back the assigned follow-up.

10 • Shift coordinator: Good. We'll close the loop with a verified concern and owner, not an assumed meaning for watcher.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 3 OF 3

A mobility handoff with an assessment still pending

Separate fictional allied-health simulation: a physiotherapy assessment has been requested but not completed; no mobility instructions are supplied.

01 • Physiotherapy assistant: The assessment request is in the queue, but the physiotherapist hasn't completed the evaluation.

02 • Receiving nurse: Thank you. I had heard that physiotherapy had already cleared the simulated patient.

03 • Physiotherapy assistant: That isn't the status I can confirm. A request is not a completed assessment.

04 • Receiving nurse: Then I shouldn't infer functional status or change any plan from that message.

05 • Physiotherapy assistant: Correct. Please use the existing authorized plan and direct clinical questions to the responsible clinician.

06 • Receiving nurse: Who will communicate the assessment outcome when it becomes available in the exercise?

07 • Physiotherapy assistant: I'll confirm the assigned physiotherapist and ask them to provide the clinical handoff.

08 • Receiving nurse: I'll keep the assessment marked pending and make that clear to the next shift.

09 • Physiotherapy assistant: Please also correct the earlier clearance message so it doesn't continue circulating.

10 • Receiving nurse: I will. I'll distinguish the requested assessment, its eventual completion, and any separately confirmed clinical recommendations.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Shift Handoffs and Clinical Prioritization

In a simulated handoff, the outgoing clinician has observations and background notes, but a requested result is still pending. The receiving clinician needs a clear transfer of information.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are taking over the work. Ask what remains open and repeat back the critical status or responsibility. Point out one detail that would be unsafe or misleading to assume.

Second round: The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

Model for scenario 1: The result is still pending. I'll separate the current situation, relevant background, my assessment, and the request in this handoff. Please repeat back the outstanding task so we can confirm understanding.

Scenario 2 | An owner missing from a pending review

A simulated handoff lists an occupational-therapy review as pending. The outgoing clinician knows it was requested but has not confirmed who accepted it. Practice transferring the information while resolving ownership; no clinical findings are supplied.

Role A: Outgoing nurse: identify the pending review and missing owner.

Role B: Incoming occupational therapist: ask for the request reference and agree how acceptance will be confirmed.

Possible opening: The review was requested, but I cannot yet confirm which clinician accepted it.

Success checks

- Distinguish a request from accepted responsibility.
- Do not invent findings or a clinical plan.
- Read back the agreed ownership check and follow-up route.

Add a complication: The next shift assumes pending means someone is already following it up.

LESSON 02 / 05 • CONVERSATIONS / 1 OF 3

Requesting review for a reported change

Fictional clinical simulation: at 10:00 case C2 spoke in full sentences; at 10:20 the patient said I feel worse and paused repeatedly while answering. No vital-sign values, diagnosis, or treatment instruction is supplied; the local escalation process is available.

01 • Bedside nurse: I'm requesting review of C2 now. At ten twenty, the patient said, I feel worse.

02 • Charge nurse: What did you observe, separately from the patient's own description of the change?

03 • Bedside nurse: At ten, they spoke in full sentences. At ten twenty, they paused repeatedly while answering.

04 • Charge nurse: Do you have vital-sign readings or a confirmed explanation for that change?

05 • Bedside nurse: Neither is supplied here. I'm reporting the change, not assigning a diagnosis.

06 • Charge nurse: I'll receive the concern and use our local escalation process to reach the responsible clinician.

07 • Bedside nurse: Please confirm who accepts the review request; I don't want the contact left unacknowledged.

08 • Charge nurse: Understood: worsening reported and repeated pauses observed at ten twenty, compared with full sentences at ten.

09 • Bedside nurse: Correct. No review outcome or rapid-response activation has been confirmed in this conversation.

10 • Charge nurse: I'll coordinate the next contact through the local process and confirm the receiving clinician.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 2 OF 3

A callback without a confirmed receiver

Separate fictional escalation rehearsal: a nurse left a message about a change, but no clinician has acknowledged receipt; local escalation instructions are available.

01 • Simulation nurse: I left a message about the change, but no one has acknowledged receiving it.

02 • Clinical supervisor: Then the communication loop is still open. What does the local escalation process direct next?

03 • Simulation nurse: I'll check and follow that process instead of assuming the message has been acted on.

04 • Clinical supervisor: Make your request explicit when you reach the next responsible person.

05 • Simulation nurse: I need a clinician to review the reported change and confirm that they have accepted the request.

06 • Clinical supervisor: Yes. Include the actual observations and timing supplied in the exercise, without adding findings.

07 • Simulation nurse: Could I describe the lack of acknowledgment as proof that the clinician ignored me?

08 • Clinical supervisor: No. It establishes an unconfirmed response, not the person's intention or what they heard.

09 • Simulation nurse: I'll restate the concern through the escalation route and ask for a clear acknowledgment.

10 • Clinical supervisor: Good. We'll record the contacts and verified response while keeping the clinical concern visible.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 3 OF 3

An unexpected monitor value needs a clear review request

Separate fictional measurement simulation: the training monitor displayed a pulse of 78 beats per minute at 09:00 and 108 at 09:20. Measurement reliability and clinical significance are unassessed; these numbers are not action thresholds or treatment guidance.

01 • Allied health clinician: I'm requesting a nursing review: the training monitor's displayed pulse changed between nine and nine twenty.

02 • Registered nurse: Please give the exact readings and distinguish the display from a confirmed clinical finding.

03 • Allied health clinician: It displayed seventy-eight beats per minute at nine and one hundred eight at nine twenty.

04 • Registered nurse: Have you established whether that reflects the patient or an issue with measurement quality?

05 • Allied health clinician: No. The explanation is unconfirmed; I haven't dismissed it as equipment error or diagnosed deterioration.

06 • Registered nurse: I'll assess the concern within my role and follow our local clinical process.

07 • Allied health clinician: Could you read back the values so I can confirm that I communicated them clearly?

08 • Registered nurse: Seventy-eight at nine, one hundred eight at nine twenty; reliability and significance still need assessment.

09 • Allied health clinician: Correct. Those are displayed readings, and I'm asking for review rather than prescribing a response.

10 • Registered nurse: Request received. I'll confirm the appropriate follow-up through the clinical process.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Patient Assessment and Escalation

During a simulation, a patient reports feeling worse and the learner identifies a change from the earlier observation. The local escalation process is available.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You can help but need a clear request and its purpose. Ask what is required and whether the timing is fixed or negotiable.

Second round: The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

Model for scenario 1: I'm concerned because this is a change from the earlier observation. I need an assessment through our escalation process now. Here is what I observed and when it changed.

Scenario 2 | A concern interrupted during a call

In a clinical simulation, a learner starts reporting a change from baseline, but the call is interrupted before the request is acknowledged. No review has been confirmed. Practice reopening the call and stating the action needed using the local escalation route.

Role A: Bedside clinician: restate the observed concern and explicit review request.

Role B: Receiving charge nurse: confirm what you heard and clarify responsibility.

Possible opening: Our call ended before I confirmed that my request for review had been received.

Success checks

- State that review is requested, not merely that information exists.
- Use only supplied observations and avoid diagnosing the cause.
- Obtain acknowledgment and follow the local process for unresolved concerns.

Add a complication: The receiver initially thinks you are calling only with a routine status update.

LESSON 03 / 05 • CONVERSATIONS / 1 OF 3

An allergy account that differs from the record

Fictional medication-reconciliation simulation: the chart says no known drug allergies. The patient reports a rash after penicillin years ago but cannot recall the date; prior verification and causality are unknown. No medication or treatment decision is supplied.

01 • Registered nurse: The chart says no known drug allergies, but the patient reports a rash after penicillin years ago.

02 • Clinical pharmacist: Did the patient give a date, and has that reported reaction been assessed previously?

03 • Registered nurse: They couldn't recall the date. I don't know whether either source was previously verified.

04 • Clinical pharmacist: Then preserve the reported rash and the chart entry separately; neither resolves the discrepancy by itself.

05 • Registered nurse: Should I describe penicillin as a confirmed cause of the rash in the handoff?

06 • Clinical pharmacist: No. Say the patient reported a rash after it; causality remains unassessed here.

07 • Registered nurse: I'm requesting reconciliation through our clinical process, not proposing a change to the MAR myself.

08 • Clinical pharmacist: Request received: charted negative history, reported penicillin-associated rash, date unknown, verification pending.

09 • Registered nurse: That's accurate. Please confirm the responsible review and communicate any authorized clarification back to the team.

10 • Clinical pharmacist: I will coordinate that follow-up and keep the allergy discrepancy visible until it is reviewed.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 2 OF 3

Unknown is not the same as no known allergies

Separate fictional chart review: an imported allergy field is blank; no patient interview or reconciliation has been completed.

01 • Medication reconciliation nurse: The imported allergy field is blank. Can I read that as no known allergies?

02 • Pharmacist: No. In this case, a blank field doesn't establish that an allergy history was checked.

03 • Medication reconciliation nurse: So the correct communication is allergy status unknown, pending the appropriate clarification?

04 • Pharmacist: Yes. State what is missing and who needs to verify it through the clinical process.

05 • Medication reconciliation nurse: Could the imported MAR help, or would that automatically resolve the uncertainty?

06 • Pharmacist: It may provide context, but an imported medication record isn't proof that allergy reconciliation occurred.

07 • Medication reconciliation nurse: I'll ask the responsible clinician to confirm the source and status rather than fill the gap myself.

08 • Pharmacist: Please use closed-loop communication so the clarification request is acknowledged.

09 • Medication reconciliation nurse: I'll read back the unresolved item and confirm the receiving clinician's responsibility for follow-up.

10 • Pharmacist: Good. Keep unknown information distinct from a verified negative history throughout the simulation.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 3 OF 3

A discussion mistaken for a medication instruction

Separate fictional medication-communication rehearsal: a colleague discussed a possible contraindication, but no reviewed instruction has been issued.

01 • Ward nurse: I heard a discussion about a possible contraindication. Was that a confirmed medication instruction?

02 • Clinical pharmacist: Not in the information supplied here. A discussion and an authorized instruction are different things.

03 • Ward nurse: Could you clarify who is reviewing the concern and where the confirmed instruction will appear?

04 • Clinical pharmacist: I'll verify the responsible reviewer through the established clinical process rather than invent an owner.

05 • Ward nurse: I don't want an informal remark to become a change in the MAR.

06 • Clinical pharmacist: Agreed. Preserve the concern, but don't represent it as a completed assessment or approved change.

07 • Ward nurse: What should I say in the meantime when another nurse asks about the conversation?

08 • Clinical pharmacist: Say a possible contraindication was raised and the reviewed instruction has not been confirmed.

09 • Ward nurse: I'll use that wording and request acknowledgment when the authorized clarification is available.

10 • Clinical pharmacist: I'll help close the loop so the team receives a verified message rather than a chain of assumptions.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Medication Safety and Allergy Clarification

A fictional medication record and the patient's account contain different allergy information. The learner must communicate the discrepancy through the clinical process.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to know exactly what information is missing. Give one answer only after your partner asks a focused question; then ask them to confirm their understanding.

Second round: The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

Model for scenario 1: The recorded allergy information differs from what the patient has told me. Could we verify the details through the appropriate clinical process? I will state the discrepancy clearly rather than assume either account is complete.

Scenario 2 | An unreadable allergy entry

In a fictional charting simulation, an allergy entry contains an unreadable abbreviation and has not been reconciled. No substance or reaction is confirmed. Practice requesting clarification without guessing the abbreviation or making a medication decision.

Role A: Registered nurse: identify the exact unreadable entry and request source verification.

Role B: Clinical pharmacist: clarify what remains unknown and arrange the appropriate review.

Possible opening: I cannot reliably interpret this allergy abbreviation, so I need the source entry clarified.

Success checks

- Do not substitute a guessed substance or reaction.
- Distinguish the unclear entry from a confirmed allergy finding.
- Confirm the review route and use a read-back when verified information arrives.

Add a complication: A colleague offers a likely expansion from memory and asks you to use it.

LESSON 04 / 05 • CONVERSATIONS / 1 OF 3

Teach-back after a nod

Fictional education rehearsal: the approved administrative sheet directs appointment-change requests to the Scheduling Office, not the ward desk. A standardized patient has nodded during the explanation. No clinical advice is practiced.

01 • Discharge nurse: This sheet directs appointment-change requests to the Scheduling Office rather than the ward desk.

02 • Standardized patient: Yes, I see the two office names. I think I understand what you're saying.

03 • Discharge nurse: To check how clearly I explained it, whom would you contact to request a different appointment?

04 • Standardized patient: I'd call the ward desk and ask them to move the appointment for me.

05 • Discharge nurse: I wasn't clear enough: for an appointment change, contact the Scheduling Office, as shown here.

06 • Standardized patient: So the ward desk isn't the contact for that request on this sheet?

07 • Discharge nurse: Correct. Could you explain the appointment-change step again in your own words?

08 • Standardized patient: If I need another appointment time, I'll contact the Scheduling Office to request a change.

09 • Discharge nurse: Exactly. You've identified the right office; a request doesn't itself confirm a new appointment.

10 • Standardized patient: Understood. I'll request the change there and wait for confirmation of any new time.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 2 OF 3

Repeating the words versus explaining the next step

Separate fictional teach-back simulation: an approved administrative sheet says to collect an attendance certificate from Reception after check-in, not before. The standardized patient can repeat the heading but has not explained the sequence.

01 • Standardized patient: I remember the heading: attendance certificate collection. Does that mean I should collect it first?

02 • Discharge nurse: The sheet says to check in first, then collect the certificate from Reception.

03 • Standardized patient: I can repeat those words, but I was unsure whether collection comes before check-in.

04 • Discharge nurse: Thank you for checking. Repeating a heading and understanding the sequence aren't necessarily the same.

05 • Standardized patient: Could you ask me a practical question so I can check that I've understood?

06 • Discharge nurse: Certainly. When you arrive, what will you do before collecting your attendance certificate?

07 • Standardized patient: I'll check in first, then go to Reception to collect the certificate.

08 • Discharge nurse: That's right. You have both the order and the collection place correct.

09 • Standardized patient: So I won't go straight to certificate collection before completing check-in.

10 • Discharge nurse: Exactly. That matches this sheet, and I'm glad we clarified the sequence together.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 3 OF 3

An unfamiliar word on the discharge sheet

Separate fictional allied-health education rehearsal: the approved administrative sheet lists a follow-up appointment for Tuesday at 09:00, with check-in at Reception. The standardized patient does not understand follow-up; no clinical instruction is practiced.

01 • Occupational therapist: Which words on this approved sheet would you like me to explain before we finish?

02 • Standardized patient: I am unsure about follow-up. Does it mean I should arrive today without an appointment?

03 • Occupational therapist: It means a later check or contact. Your listed appointment is Tuesday at nine, with check-in at Reception.

04 • Standardized patient: So follow-up is not an instruction to come immediately; the appointment details tell me when?

05 • Occupational therapist: Correct. To check my explanation, could you tell me when and where you will check in?

06 • Standardized patient: I will check in at Reception on Tuesday at nine for the listed appointment.

07 • Occupational therapist: Exactly. That matches the time and place on your sheet, and you explained it clearly.

08 • Standardized patient: If another detail is unclear, should I guess rather than ask the team again?

09 • Occupational therapist: Please ask for clarification. We can identify the right contact without inventing an answer.

10 • Standardized patient: Thank you. I understand the appointment arrangement and will ask about anything still unclear.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Patient Education and Teach-Back

A patient in a role-play nods during an explanation of an approved discharge sheet. The learner needs to check whether the explanation was clear.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are unfamiliar with one technical term in the case. Ask for a plain-English explanation and then paraphrase it. Do not pretend to understand a term you cannot explain.

Second round: Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Model for scenario 1: I want to check that I explained this clearly. In your own words, how will you follow the instructions when you get home? We can go over any part that is unclear.

Scenario 2 | A translation request before teach-back

In a fictional education exercise, the standardized patient says the approved discharge sheet is in a language they cannot read comfortably. An appropriate language-support request route is available, but support has not yet been arranged.

Role A: Discharge nurse: acknowledge the language need and request appropriate support without improvising clinical translation.

Role B: Interpreter coordinator: clarify the requested language and explain that availability still needs confirmation.

Possible opening: Before checking understanding, I need to arrange appropriate language support for this explanation.

Success checks

- Distinguish language access from demonstrated understanding.
- Do not invent interpreter availability or translate unsupplied clinical content.
- Agree the support request and a later teach-back check using approved information.

Add a complication: A colleague suggests treating the patient's polite nod as sufficient understanding.

LESSON 05 / 05 • CONVERSATIONS / 1 OF 3

Making room for the mobility concern

Fictional interprofessional rounds: discharge timing is being discussed, while a mobility concern remains unresolved.

01 • Rounds coordinator: Before we discuss a discharge time, let's hear the unresolved mobility concern from physiotherapy.

02 • Physiotherapist: Thank you. The concern needs assessment, and I don't want the timing discussion to imply clearance.

03 • Rounds coordinator: What information does the team need before the responsible clinician considers the plan of care?

04 • Physiotherapist: The current functional-status assessment and any remaining questions, without assuming the outcome in advance.

05 • Rounds coordinator: Case management needs timing information too. Can we identify that dependency without deciding discharge here?

06 • Physiotherapist: Yes. Say timing remains dependent on the relevant clinical review rather than promise a departure.

07 • Rounds coordinator: I'll invite case management to explain its coordination needs after your clinical concern is heard.

08 • Physiotherapist: Please also clarify who has authority to confirm the discharge plan in this simulation.

09 • Rounds coordinator: I'll verify that role and summarize the unresolved concern before seeking a decision.

10 • Physiotherapist: Good. The group can coordinate a safe discharge discussion without treating silence or schedule pressure as clinical agreement.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 2 OF 3

Equipment requested but not confirmed

Separate fictional rounds exercise: an occupational-therapy equipment request is pending, while case management is preparing discharge logistics.

01 • Case manager: I'm organizing the logistics, but I need to know whether the equipment request is confirmed.

02 • Occupational therapist: The request is pending. We haven't received confirmation of availability or the final arrangement.

03 • Case manager: Could I list equipment ready and let the receiving team check it later?

04 • Occupational therapist: That would hide the unresolved dependency. Please keep requested and confirmed as separate statuses.

05 • Case manager: Let's bring the responsible supply coordinator into the discussion before we settle the logistics.

06 • Occupational therapist: Agreed. I'll explain the assessment-related request without implying a new clinical recommendation here.

07 • Case manager: Who should confirm the final plan of care once the relevant information is available?

08 • Occupational therapist: The authorized clinical team; our logistical discussion doesn't replace that decision.

09 • Case manager: I'll summarize the pending equipment question and assign the availability check for confirmation.

10 • Occupational therapist: I'll review the returned information with the team so we can distinguish coordination progress from clinical readiness.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 3 OF 3

Different meanings of ready during rounds

Separate fictional rounds rehearsal: nursing says education is ready to deliver, while the meeting chair interprets ready as all discharge work completed.

01 • Meeting chair: You said ready. Are all parts of the simulated discharge plan complete now?

02 • Registered nurse: I meant the education materials are ready to deliver, not that education or discharge is complete.

03 • Meeting chair: Thank you for correcting that. Which other voices should we hear before discussing readiness?

04 • Registered nurse: Ask each relevant discipline to state its own status instead of letting my comment stand for the team.

05 • Meeting chair: I'll invite case management and therapy to identify completed work and unresolved dependencies.

06 • Registered nurse: Please separate information sharing from the authorized decision about the plan of care.

07 • Meeting chair: Agreed. I won't record team agreement merely because nobody interrupted the word ready.

08 • Registered nurse: I can report the education step accurately after it occurs and its understanding check is completed.

09 • Meeting chair: For now, I'll summarize materials prepared, education pending, and other disciplines' status awaiting confirmation.

10 • Registered nurse: That gives the team a clear starting point without turning one completed preparation task into a discharge decision.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Interprofessional Rounds

In simulated rounds, one team member focuses on discharge timing and another raises an unresolved mobility concern.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You hold a relevant concern that has not been discussed. Raise it when invited and ask who will decide. Help the facilitator state one unresolved question accurately.

Second round: A participant interrupts or the meeting is running out of time. Protect a turn to speak and close with an accurate decision record.

Model for scenario 1: Before we agree on timing, could we hear the mobility concern and the current assessment? Let's confirm the outstanding questions and who will address them in the care plan.

Scenario 2 | A missing discipline at the huddle

In a simulated discharge huddle, the speech and language therapist is absent and their review is still pending. The group wants to finalize coordination arrangements. No assessment outcome is supplied. Facilitate a bounded discussion of the missing input.

Role A: Rounds facilitator: invite available perspectives and identify the missing review.

Role B: Case manager: explain logistical needs without substituting for the absent clinician.

Possible opening: We can discuss coordination needs, but the pending review must remain visible before an authorized decision.

Success checks

- Reject silence as evidence of clinical agreement.
- Separate logistics from the missing clinical assessment.
- Assign a contact owner and clarify who can confirm the plan.

Add a complication: A participant says the absent therapist's silence means agreement.

LESSON 06 / 05 • CONVERSATIONS / 1 OF 3

Replace difficult with an observed account

Fictional charting exercise: a draft note says difficult but includes no observed behavior or patient quotation.

01 • Clinical preceptor: This simulated note calls the patient difficult. What did you actually observe or hear?

02 • New nurse: The draft doesn't include that information, so I can't support the label with an objective finding.

03 • Clinical preceptor: Could you replace it with uncooperative and keep the rest of the note?

04 • New nurse: That would still be an interpretation without a described event or patient response.

05 • Clinical preceptor: What do you need before revising the charting exercise accurately and usefully?

06 • New nurse: The actual observation, relevant words, and any documented intervention, without inventing a behavioral example.

07 • Clinical preceptor: Good. A neutral description should help the reader understand the event rather than judge the person.

08 • New nurse: I'll ask for the missing source details and distinguish quotations from my own observations.

09 • Clinical preceptor: Also keep the effect of any intervention separate from what you hoped it would achieve.

10 • New nurse: Understood. I won't replace an unsupported label with a fabricated fact just to make the note look complete.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 2 OF 3

A request for more explanation is not refusal

Separate fictional documentation simulation: the patient said, 'Please explain that again before we continue'; no further response is supplied.

01 • Staff nurse: The simulated patient asked me to explain again before continuing. Can I chart refused?

02 • Nurse educator: That wording goes beyond the supplied statement. They requested another explanation, not necessarily refused.

03 • Staff nurse: Would quoting the actual sentence preserve the distinction more accurately in the record?

04 • Nurse educator: Yes. Identify it as the patient's statement and describe any actual subsequent action separately.

05 • Staff nurse: The exercise doesn't say whether I explained again or whether the patient then agreed.

06 • Nurse educator: Then neither outcome should appear as a completed intervention or patient response.

07 • Staff nurse: I'll keep the unknown sequence visible rather than supply a likely ending for the chart.

08 • Nurse educator: Good. Objective charting can contain uncertainty when the source information is incomplete.

09 • Staff nurse: I'll ask for the remaining simulation details before documenting anything beyond the quotation.

10 • Nurse educator: That preserves what was said without turning a communication request into an unsupported judgment about cooperation.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 3 OF 3

Observed later does not mean caused by the intervention

Separate fictional note review: an intervention is documented and a later patient response is recorded, but the relationship has not been assessed.

01 • Allied health clinician: My note says the intervention caused the later improvement. Is that supported by the sequence alone?

02 • Documentation mentor: The sequence establishes that one happened before the other, not necessarily why the response changed.

03 • Allied health clinician: Should I remove the patient response completely to avoid making a causal claim?

04 • Documentation mentor: No. Preserve the observed response and its timing, while qualifying any interpretation of its cause.

05 • Allied health clinician: The simulation gives no additional assessment, so I shouldn't invent a clinical explanation.

06 • Documentation mentor: Correct. Distinguish the recorded intervention, the objective finding, and any assessment actually made.

07 • Allied health clinician: Could I say the response was observed after the intervention without saying caused by?

08 • Documentation mentor: That is a more accurate temporal description, provided it matches the supplied record.

09 • Allied health clinician: I'll revise the causal wording and retain the source observations for the reader.

10 • Documentation mentor: Good. The note can be useful without claiming a relationship that this exercise hasn't established.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Documentation and Charting

A draft simulated note says a patient was "difficult." It contains no description of what was said or observed.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Second round: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Model for scenario 1: Let's replace that label with observable information. Record what the patient said or did, the relevant context, the action taken, and the response, following the documentation process.

Scenario 2 | A colleague's observation in your note

In a fictional charting simulation, another clinician tells you a patient declined a conversation. You did not witness it, and the exact words are unknown. Discuss how to communicate the source and uncertainty before any documentation is finalized.

Role A: Receiving clinician: distinguish secondhand information from your own observation.

Role B: Reporting clinician: clarify what you actually observed without adding motives.

Possible opening: I did not witness the conversation, so I need to distinguish your report from my own findings.

Success checks

- Attribute the report to its actual source.
- Avoid inventing quotations, emotion, or motivation.
- Agree which observed details need clarification through the documentation process.

Add a complication: A colleague suggests writing the patient was angry, although no behavior supporting that description is supplied.

LESSON 07 / 05 • CONVERSATIONS / 1 OF 3

Responding to nobody tells us anything

Fictional family-communication rehearsal between a nurse and a patient liaison: a family member has said nobody tells us anything; sharing permissions are unconfirmed.

01 • Patient liaison: The family says nobody tells us anything. How will you respond without becoming defensive?

02 • Registered nurse: I'll acknowledge their frustration and explain that I can help arrange an appropriate conversation.

03 • Patient liaison: Would you promise to disclose the full clinical record to repair the communication problem?

04 • Registered nurse: No. I need to respect privacy and confirm what information can appropriately be shared.

05 • Patient liaison: How can you explain that boundary without sounding as though you're refusing to help?

06 • Registered nurse: I can help identify the right clinician and arrange a discussion within the applicable sharing arrangements.

07 • Patient liaison: The family may ask you for a decision outside your scope of practice.

08 • Registered nurse: I'll explain my role clearly and refer that question to the responsible clinician rather than guess.

09 • Patient liaison: Please avoid promising a family meeting time before the participants are available.

10 • Registered nurse: Agreed. I'll take responsibility for requesting the conversation and returning a confirmed arrangement.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 2 OF 3

Correcting an unconfirmed family-meeting time

Separate fictional communication case: a nurse said the family meeting would be at two, but clinician availability has not been confirmed.

01 • Nurse coordinator: I told the family two o'clock, but I hadn't confirmed the clinician's availability.

02 • Social worker: We need to correct that promise directly rather than leave them waiting for an unconfirmed meeting.

03 • Nurse coordinator: I'll say I gave a time before confirming it, and I'm sorry for that confusion.

04 • Social worker: Good. What can you offer now without creating another unsupported promise?

05 • Nurse coordinator: I can request a confirmed time and remain the contact for the scheduling update.

06 • Social worker: Should the correction also clarify the purpose of the proposed family meeting?

07 • Nurse coordinator: Yes, within the agreed scope and privacy arrangements, without promising a specific clinical decision.

08 • Social worker: I'll help check who needs to attend and what questions the family wants addressed.

09 • Nurse coordinator: I'll distinguish collecting their questions from guaranteeing that every requested outcome can be provided.

10 • Social worker: Then we'll return with a verified arrangement and a clear account of who will address which concerns.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 3 OF 3

An apology without crossing a privacy boundary

Separate fictional liaison call: an earlier message implied any relative could receive updates, but the appropriate sharing permissions have not been verified.

01 • Patient liaison: Our earlier message suggested any relative could receive clinical updates. That needs correction.

02 • Charge nurse: I'll acknowledge the misleading wording and explain that appropriate sharing permissions must be checked.

03 • Patient liaison: Can you make that correction without implying the relative has done something wrong?

04 • Charge nurse: Yes. The problem was our explanation; the check is about information sharing, not a judgment about them.

05 • Patient liaison: What should happen if they ask you to confirm details while the check is pending?

06 • Charge nurse: I'll maintain the privacy boundary and help them reach the appropriate conversation route.

07 • Patient liaison: Could you promise that permission will be granted once the form is reviewed?

08 • Charge nurse: No. Review is not a guaranteed outcome, and I shouldn't imply authority I don't have.

09 • Patient liaison: I'll support the follow-up and make sure the family receives a clear explanation of the next step.

10 • Charge nurse: Thank you. We can repair the message and remain helpful without making an unverified disclosure.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Difficult Families and Boundaries

A family member in a role-play says, "Nobody tells us anything." The learner can explain their role and arrange an appropriate conversation.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are frustrated because an earlier message created an expectation. Explain that expectation. Ask your partner to distinguish the correction from anything still uncertain.

Second round: The listener remains disappointed after the correction. Acknowledge the impact and restate the available next step calmly.

Model for scenario 1: I can hear that the lack of information is frustrating. Let me explain what I can clarify and arrange contact with the appropriate team member for the questions outside my role.

Scenario 2 | A question redirected without explanation

In a fictional family liaison exercise, a relative was transferred between two departments without being told why. Their clinical question remains unanswered, and the responsible clinician has not been identified. Practice a repair conversation between staff.

Role A: Patient liaison: acknowledge the confusing transfers and ask for a named contact route.

Role B: Ward coordinator: clarify your own role and arrange identification of the appropriate clinician.

Possible opening: We transferred the question without explaining the route, so we need to give the family a clearer next step.

Success checks

- Acknowledge the communication failure without blaming the relative.
- Keep the response within role and privacy boundaries.
- Agree who identifies and confirms the appropriate contact.

Add a complication: A colleague proposes answering the clinical question from memory to end the complaint.

LESSON 08 / 05 • CONVERSATIONS / 1 OF 3

A fall report without a blame conclusion

Fictional safety-report review: a fall is reported, but contributing factors are unestablished; a draft calls a colleague careless.

01 • Safety facilitator: The incident report describes a fall and then calls a colleague careless. What supports that conclusion?

02 • Staff nurse: Nothing in the supplied account establishes carelessness or the contributing factors.

03 • Safety facilitator: What should remain in the report while the review gathers additional information?

04 • Staff nurse: The observed event, available timing, and known actions, with unverified details clearly identified.

05 • Safety facilitator: Would removing the blame word mean we are avoiding accountability for the event?

06 • Staff nurse: No. A just-culture review can examine responsibilities without treating an unsupported accusation as a finding.

07 • Safety facilitator: Should you name a root cause now so the exercise has a definite ending?

08 • Staff nurse: No. We can end with an assigned review step while the cause remains unknown.

09 • Safety facilitator: Please also distinguish any recorded fall-risk information from a conclusion about why this fall happened.

10 • Staff nurse: I will. I'll preserve the available evidence and send the unresolved questions through the incident-review process.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 2 OF 3

A missing entry after an intercepted mix-up

Separate fictional safety simulation: a label mix-up was intercepted before use; the check's documentation is missing, and the sequence is not fully established.

01 • Clinical safety reviewer: The mix-up was intercepted before use, but the check isn't documented in the supplied record.

02 • Ward supervisor: Can we conclude that the check was skipped and that this caused the mix-up?

03 • Clinical safety reviewer: No. Missing documentation and a demonstrated causal sequence are different findings.

04 • Ward supervisor: What questions should we ask without leading colleagues toward a preferred explanation?

05 • Clinical safety reviewer: Ask what each person saw, what actions occurred, and which records can establish the sequence.

06 • Ward supervisor: Should we omit the event because the interception prevented the intended use?

07 • Clinical safety reviewer: No. Follow the simulation's reporting process; an intercepted event can still provide useful safety information.

08 • Ward supervisor: I'll describe the known interception and keep the undocumented step marked unresolved.

09 • Clinical safety reviewer: I'll gather the available accounts without treating a single explanation as the root cause.

10 • Ward supervisor: Then our discussion can identify follow-up responsibly without claiming either blame or complete system safety.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 3 OF 3

One refresher session does not prove prevention

Separate fictional safety review: staff attended a refresher after an incident; no effectiveness assessment or root-cause finding has been completed.

01 • Education coordinator: Everyone attended the refresher. Can we report that the incident's root cause is resolved?

02 • Safety lead: No. Attendance confirms participation, while the cause and effectiveness assessment remain open.

03 • Education coordinator: The session covered fall-risk communication. Isn't that enough to show future falls are prevented?

04 • Safety lead: It doesn't establish that outcome. We shouldn't turn a training activity into a universal prevention claim.

05 • Education coordinator: How do we give a positive but accurate update on what the team completed?

06 • Safety lead: State that the refresher occurred and distinguish it from the pending review and effectiveness work.

07 • Education coordinator: Could staff interpret that distinction as saying their participation didn't matter at all?

08 • Safety lead: Explain its purpose, while being clear about what evidence it does and doesn't provide.

09 • Education coordinator: I'll describe the completed session and keep the incident report linked to the ongoing review.

10 • Safety lead: Good. We'll evaluate the relevant evidence before closing actions or claiming the contributing factors have been addressed.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Safety Events and Just Culture

A simulated safety report includes a fall and an assumption that a colleague was careless. The review has not established the contributing factors.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Second round: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Model for scenario 1: The report should describe the event and available observations. We have not established the contributing factors. I'll remove the unsupported judgment and use the reporting process to raise the concern.

Scenario 2 | Two accounts of the same safety event

In a fictional incident review, two clinicians remember a call occurring at different times. Neither timestamp has been verified, and no causal conclusion exists. Discuss how to preserve both accounts and seek corroboration.

Role A: Safety interviewer: describe the disagreement neutrally and request available records.

Role B: Clinical supervisor: separate recollection from verified timing and keep the review open.

Possible opening: The accounts differ on the call time, so we need to preserve both while checking the records.

Success checks

- Do not treat seniority as proof that an account is correct.
- Distinguish recalled timing from verified evidence.
- Agree a corroboration step without assigning blame or a root cause.

Add a complication: A participant says the newer clinician's account should be discarded solely because they have less experience.

Give feedback that helps

Score each criterion 0, 1, or 2. This is a practice rubric, not a professional qualification or CEFR test. Do not award or remove points for a particular accent.

Meaning and accuracy

Keeps the case facts accurate; clearly separates confirmed and unknown information.

Clarity and language

Uses understandable sentences and explains specialist terms when the listener needs it.

Interaction and tone

Responds to the other person's question, checks understanding, and uses an appropriate tone.

Task completion

Makes the requested action or unresolved question clear without inventing authority or facts.

Use the same scale for each criterion

0 - Not yet: The reader or listener cannot yet recover the intended meaning or action.

1 - With support: The message works after a prompt, clarification, or revision.

2 - Independently: The message is clear and accurate without a prompt.

Feedback sequence

First, identify a successful phrase. Next, identify one point where meaning or accuracy needs work. Offer one useful language correction, allow a repeat, and compare the two attempts. A total out of eight describes this performance only; do not translate it into a proficiency level.

Final challenge

Choose a case not rehearsed today. The learner gives a one-minute response, answers two follow-up questions, and writes a 70-110 word message. Add the lesson's second-round challenge. Compare the result with an earlier attempt using the four criteria.

Use the language responsibly

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

AHRQ: SBAR communication tool

<https://www.ahrq.gov/teamstepps-program/curriculum/communication/tools/sbar.html>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

Extend your practice with AI

Use the next two prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Writing feedback begins with your draft. Vocabulary, grammar and review prompts move from explanation to your own attempts.

Choose any lesson or dialogue online

The course prompt workshop has eight practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

[Open this course's AI prompt workshop](#)

Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

The two starter prompts use B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-28. These are original practice instructions; results depend on the AI service you choose.

Rehearse with an AI colleague

START PROMPT / COPY THROUGH END PROMPT

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Nursing and Allied Health English

Professional audience: nurses, charge nurses, physical therapists, occupational therapists, respiratory therapists, imaging staff, laboratory staff, care-team coordinators, and allied-health supervisors

Scope: Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Dialogue 01: Shift handoff with a pending result

Original fictional setting: Two nurses transfer care using the approved handoff process.

Professional roles: Outgoing nurse / Incoming nurse

Published dialogue (reference script, not my own performance):

Outgoing nurse: For the patient we've identified in the chart, the repeat lab result is still pending. The care team has been notified.

Incoming nurse: What prompted the repeat, and what follow-up has been requested?

Outgoing nurse: The initial sample was unsuitable. The record includes the collection time and the clinician's current plan.

Incoming nurse: I'll verify that plan in the chart. Who is checking for the new result?

Outgoing nurse: I'm handing that follow-up to you now. Please acknowledge it in our handoff record.

Incoming nurse: Understood. I'll check the result and escalate according to the documented plan and local procedure.

END REFERENCE

TASK: Interactive workplace role-play

1. Use the supplied case or dialogue as the starting situation. Give a two-sentence briefing and offer two relevant professional roles for me to choose from. For a supplied dialogue, use its actual roles. Ask which role I want, then stop and wait. Do not write my replies.
2. After I choose, identify who you will play and the immediate communication goal. Play the other professional; where a meeting requires a third person, label each of your speakers clearly. Start with one natural workplace turn and wait for my reply. Keep most turns to one to three sentences and ask no more than one question at a time.
3. Let the exchange develop across six to ten learner turns, or end earlier if I type "feedback". Respond to what I actually say. Include a plausible clarification, disagreement or tradeoff without silently changing the starting facts. Mark any added constraint as a fictional second-round variation. Do not resolve approvals, evidence or commitments that remain uncertain.
4. Stay in role during the exchange. If my meaning is unclear, ask for clarification naturally. Give a hint only if I ask or cannot proceed. Do not deliver a model conversation in advance.
5. At the debrief, quote two of my phrases: one successful choice and one worth improving. Check factual accuracy, language, register and whether the next step was clear. Give up to three focused suggestions. Ask me to retry the weakest turn; wait. Only then offer an alternative wording and a harder replay.

END PROMPT

Copy this prompt online or choose another lesson and support level

Create more scenario dialogues

START PROMPT / COPY THROUGH END PROMPT

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

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Incoming nurse: I'll verify that plan in the chart. Who is checking for the new result?

Outgoing nurse: I'm handing that follow-up to you now. Please acknowledge it in our handoff record.

Incoming nurse: Understood. I'll check the result and escalate according to the documented plan and local procedure.

END REFERENCE**TASK:** Extend the conversation library

1. Propose six distinct, common scenarios in this field beyond the supplied case or script. For each, name the setting, two or more professional roles, the communication problem and one useful terminology focus. Include routine coordination, clarification, a complication, disagreement, handoff and follow-up where relevant. Choose scenarios that fit this profession, rather than forcing unsuitable situations into it.
2. Ask me to choose one scenario, or request all six in sequence. Stop and wait. Do not write all the scripts before I choose.
3. For the selected scenario, write an original fictional dialogue of 12-18 substantial speaking turns between two or three professionals. Name each role, establish an actual work problem, and let the exchange progress through questions, clarification, competing constraints and a credible next step or explicitly unresolved issue. Use the occupation's natural nomenclature and register. Avoid an interview between a teacher and a learner, generic small talk, and inserting a glossary definition into every reply. Explain specialist terms outside the dialogue instead.
4. After the script, explain five useful expressions in context, identify two grammar or register choices and ask three questions about the speakers' reasoning. Withhold the answers until I attempt them. Add one role-switch challenge.
5. Before presenting a script, check names, numbers, chronology, roles and terminology for consistency. Label invented facts and acknowledge uncertain specialist usage. If I requested all six, deliver one complete script at a time and wait for "next". Make each scenario materially different.

END PROMPT

Copy this prompt online or choose another lesson and support level